Pembrolizumab · every 3 weeks · melanoma before and after surgery

Pembrolizumab is given before and after planned surgery in this selected melanoma pathway. The team explains the expected benefit, uncertainty, surgical plan and how side effects could change the sequence.

Selected stage IIIB melanoma eligible for the NCCP adjuvant indication: the treating clinician may choose pembrolizumab 200 mg before and after surgery. This is a distinct, discretionary perioperative plan. · NCCP 00455 v16 · Document revised 7 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Selected stage IIIB melanoma eligible for the NCCP adjuvant indication: the treating clinician may choose pembrolizumab 200 mg before and after surgery. This is a distinct, discretionary perioperative plan. This information supports individual consent; it does not issue or change a prescription.

My overall treatment

Every 3 weeks: 3 doses → surgery → 15 doses.

21 day view

Treatment timing in this window

Pembrolizumab treatment — Day 1

Before surgery	Surgery and recovery	After surgery
Pembrolizumab 200 mg intravenous (IV) over 30 minutes every 3 weeks for 3 doses. The team checks safety and coordinates the surgical assessment.	Surgery has its own date, assessment and recovery plan. Confirm the postoperative treatment decision; the calendar does not automatically authorise restarting.	Pembrolizumab 200 mg IV over 30 minutes every 3 weeks for 15 postoperative doses, subject to recovery, safety and clinical review.

A typical 21-day cycle

Day 1	Day 2–21
Treatment visit	Between visits

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Baseline safety** NCCP	Blood count, kidneys/liver, glucose and thyroid.
During	**Before each dose** NCCP	Blood count, kidneys/liver and glucose. thyroid-stimulating hormone (TSH) every 3–6 weeks; may fall between visits. Cortisol if indicated.
After / response	**Follow-up** NCCP / individual plan	Report immune symptoms after stopping. Scans / examinations individually planned.

Between treatments

- Immune symptoms can occur between visits or after treatment stops. Report changes promptly.
- Carry your immunotherapy alert card; follow prescribed steroids or hormone replacement.
- A permanent-stop decision still applies after symptoms improve; your team decides whether restarting is safe.
- A permanent-stop decision still applies after symptoms improve; your team decides whether restarting is safe.

Bring the written medicine plan, test dates and actual oncology/out-of-hours contacts.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hormones / head / eyes

Pembrolizumab

New severe headache, visual change, unusual tiredness, dizziness, thirst or frequent urination may signal immune effects and needs prompt advice.

3 Heart / muscles

Pembrolizumab

Chest pain, palpitations, marked muscle weakness or swallowing difficulty can be serious immune effects. Seek urgent assessment.

5 Kidneys / urine

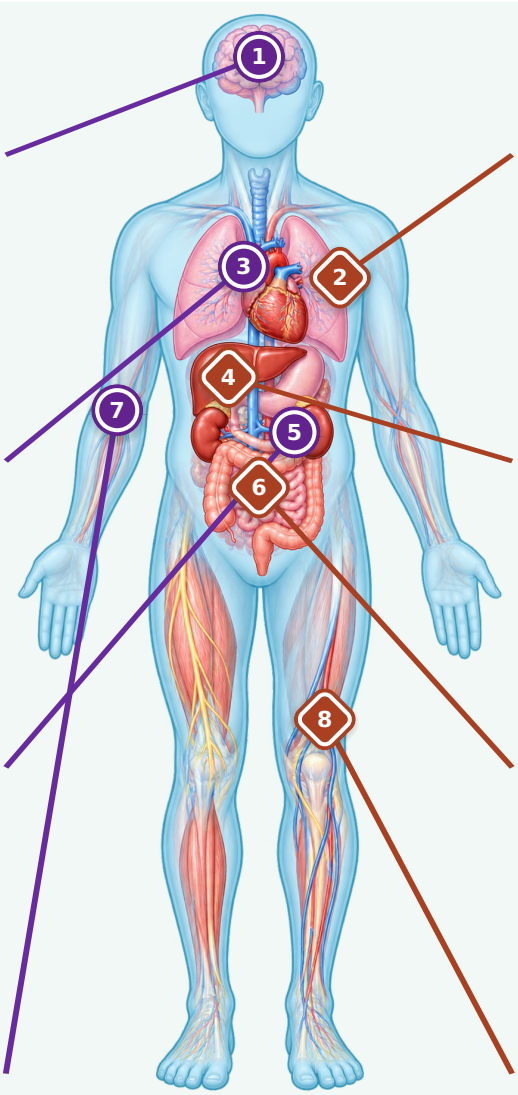
Pembrolizumab

Kidney inflammation may be detected on blood tests. Report reduced urine, swelling, blood in urine or a marked change from normal.

7 Skin / mouth

Pembrolizumab

Report rash or itching early. Blistering, peeling, mouth or genital sores, or rash with fever needs urgent assessment.



2 Lungs / breathing

Pembrolizumab

New cough or breathlessness may reflect lung inflammation or another serious cause. Call promptly; severe breathing difficulty is an emergency.

4 Liver

Pembrolizumab

Inflammation may first show in blood tests. Report yellow eyes or skin, dark urine, unusual itching or upper-abdominal pain.

6 Bowels / abdomen

Pembrolizumab

Report increased stool frequency, diarrhoea, blood or mucus, or abdominal pain early. Severe pain, black stools or dehydration needs urgent help.

8 Nerves / strength

Pembrolizumab

New tingling, weakness, unsteadiness, facial change or trouble swallowing needs prompt review. Sudden weakness or speech difficulty is an emergency.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

Keep your treatment details to hand

Selected stage IIIB melanoma eligible for the NCCP adjuvant indication: the treating clinician may choose pembrolizumab 200 mg before and after surgery. This is a distinct, discretionary perioperative plan. · NCCP 00455 v16 · Document revised 7 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Pembrolizumab · melanoma

Every 3 weeks: 3 doses → surgery → 15 doses.

Phase: _____ Before surgery: 3 doses

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

Before-surgery record

3 preoperative doses only; 15 postoperative doses in passport.

Pre 1	Pre 2	Pre 3
Date / initials	Date / initials	Date / initials

My treatment information

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.